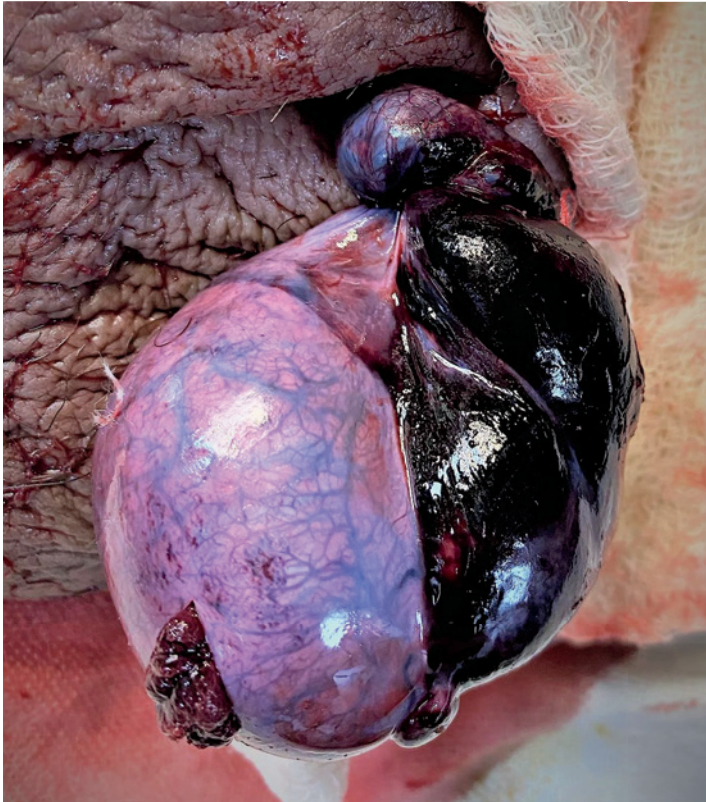


IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

Testicular Torsion



A 16-YEAR-OLD BOY WITH NO CLINICALLY SIGNIFICANT MEDICAL HISTORY presented to the emergency department with a 2-day history of left testicular pain, nausea, and vomiting. He lived in a remote area of India and said that the pain had developed suddenly while he was taking a bath. On physical examination, the left side of the scrotum was erythematous. The left testicle was tender on palpation and elevated as compared with the right testicle, and the cremasteric reflex on the left side was absent. The patient was taken to the operating room for emergency scrotal exploration. On gross examination, the left testicle was black, with 1080 degrees of spermatic cord rotation. There was no bell clapper deformity (an abnormal attachment of the tunica vaginalis that enables increased testicular mobility). Despite detorsion of the testis, there was no improvement in its color during 15 minutes of observation. An incision made in the tunica albuginea yielded no blood. Left orchiectomy and right orchiopexy were performed. The patient's pain and swelling subsided post-operatively, and he was discharged 2 days later.

DOI: 10.1056/NEJMicm2110702

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2021, at NEJM.org.